

Sample #

Gender: Female

DOB:

Race: Unknown

Report Date:

Pathology Report:



Surgical Pathology Report

FINAL PATHOLOGIC DIAGNOSIS

A. Peritoneal nodule; excision:

- Ovary with microscopic metastatic foci of urothelial carcinoma, mostly present in lymphatic channels, see comment.
- Benign ovarian epithelial cysts.

B. "Right ovary"; oophorectomy:

- Fallopian tube, no tumor.
- No ovarian tissue present.

C. Left ovary; oophorectomy:

- Ovary with stromal hyperplasia.

D. Urinary bladder and urethra; cystectomy:

- High grade urothelial carcinoma, invading thru the muscularis propria and into the perivesical tissue, with multiple lymphovascular space invasion; see pathologic parameters.

E. Left pelvic lymph nodes; dissection:

- One lymph node diffusely involved by metastatic urothelial carcinoma, with extra nodal extension (1/1).

F. Left pelvic lymph nodes; lymph node dissection:

- Four lymph nodes positive for metastatic urothelial carcinoma with soft tissue involvement (4/4).
- Largest node diffusely involved by tumor (5.5 cm), with extra nodal extension.

G. Right pelvic lymph nodes; lymph node dissection:

- Six lymph nodes positive for metastatic urothelial carcinoma with soft

tissue involvement (6/6).

- Largest lymph node tumor measures 5 cm, with extra nodal extension.

H. Left distal ureter; excision:

- Portion of ureter, no tumor.

I. Right distal ureter; excision:

- Portion of ureter, margin side free of tumor.

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Invasive urothelial carcinoma.
2. Grade of tumor: High grade.
3. Depth of invasion: Extravesicular soft tissue (macroscopic).
4. Tumor distribution: 4.5 cm tumor involving right lateral wall, left lateral wall, trigone, anterior wall and posterior wall.
5. Ureteral margins: Negative for tumor.
6. Distal urethral margin: Negative for tumor.
7. Soft tissue margin or serosa: Negative for tumor.
8. Lymph nodes: Positive for tumor (11/11), with extranodal extension.
9. pTNM: pT3b,N2,M1.

Effective January 1, 2010 this Checklist utilizes the 7th edition TNM staging system for Bladder of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

Comment

On part A, the tumor is present only on the permanent slides.

Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow.

Electronically Signed Out by ☐ MD

Clinical History:

Patient is a year-old female with bladder cancer, undergoing cystectomy, ileo conduit.

Specimens Received:

A: Peritoneal nodule

B: Right ovary

C: Left ovary

D: Bladder and urethra

E: Left pelvic lymph nodes
F: Left pelvic lymph nodes
G: Right pelvic lymph nodes
H: Left distal ureter
I: Right distal ureter

Gross Description:

The specimens are received in nine containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "peritoneal nodule". Received fresh for intraoperative diagnosis is a 2.74g, 2.3 x 1.7 x 1.3 cm fragment of white rubbery tissue that is bisected to reveal a multicystic cut surface consistent with ovary. One half of the specimen is frozen and read as "ovarian tissue, with benign cyst". The frozen section remnant is submitted in cassette A1 FS. The remainder of the specimen is entirely submitted in cassette A2.

B. The second container is additionally identified as, "right ovary". Received fresh and placed in formalin is a 4.8 x 1.2 x 0.5 cm tan-brown heavily cauterized rubbery tissue fragment. The specimen is serially sectioned to reveal fatty fibrous tissue with no gross ovary identified. Specimen is entirely submitted in cassettes B1-B4.

C. The third container is additionally identified as, "left ovary". Received fresh and placed in formalin is a 7.56 g, 3.5 x 3 x 1.2 cm firm white-tan nodule with adjacent fibrofatty tissue. The specimen is bivalved to reveal a uniform yellow-tan nodule within a 0.1 cm thick white-tan cyst wall. No gross ovarian tissue is identified. The specimen is entirely submitted in cassette C1-C4.

D. The fourth container is additionally identified as, "bladder and urethra". Received fresh and placed in formalin is a 249.06g, 15 x 13 x 4.2 cm cystectomy specimen with attached mesenteric fat and tubing in bladder. The bladder measures 7 x 6.5 x 3 cm. The urethral stump measures 3 cm in length by 1.1 cm diameter and is probe patent. The right ureteral stump measures 4 cm in length by 0.6 cm in diameter which is discontinuous and disrupted by cautery midway through the length of the ureter. The right ureter probes patent but the orifice is involved by shaggy indurated mucosa. The left ureteral stump is discontinuous to suture and cautery at one segment measuring 1 cm in length by 0.6 cm in diameter. The left ureteral orifice can't be identified at the trigone. Additionally the bladder is remarkable for a 6 x 4.5 cm portion of mucosa attached to the posterior aspect of the urethra to the lower posterior aspect of the bladder. The urethral margin is inked black.

The right half of the bladder is inked blue and the left half is inked black.
The bladder is opened along the right lateral aspect through the dome.

The bladder demonstrates an indurated, ulcerated lesion in the lower anterior-left wall, left lateral wall and posterior wall at trigone. There is an ill-defined 4.5 x 4 cm indurated area which contains a 3.5 x 3 cm ulceration surrounded by a hemorrhagic rim of mucosa. Serial sectioning reveals that the tumor grossly infiltrates into the urethra located 2.8 cm from the urethral margin and 0.8 cm from the mucosal surface. On cut section the tumor is white-tan firm, ill-defined extending a maximum depth of 1.8 cm (tumor max dimensions 4.5 x 4 x 1.8 cm) invading into adipose tissue which abuts the inked surface (black). The remainder of the mucosa is tan-brown, edematous, and glistening with a uniform 0.8-1.3 cm wall thickness. Representative sections are submitted as follows:

- D1: Urethral margin
- D2: Left ureteral margin
- D3: Right ureteral margin
- D4: Tumor relationship to urethra
- D5: Anterior wall tumor
- D6: Uninvolved anterior wall
- D7: Dome
- D8: Trigone with tumor (closest approach to inked margin)
- D9-D10: Posterior wall with tumor, hemorrhage and extension to ink
- D11: Right wall uninvolved with tumor
- D12: Left wall with tumor
- D13: Uninvolved left wall
- D14-D15: Additional representative sections of tumor
- D16: Additional section of urethra

E. The fifth container is additionally identified as, "left pelvic lymph nodes". Received fresh for intraoperative consultation is a firm tan-pink lymph node measuring 3.2 x 2.3 x 0.8 cm. One half of the specimen was frozen and read as "positive for carcinoma" by Dr. [] The frozen section remnant is submitted in cassette E1 FS. The remainder of the specimen is submitted in cassette E2.

F. The sixth container is additionally identified as, "left pelvic lymph nodes". Received fresh and placed in formalin is a 15.92 g, 6.2 x 4 x 1.2 cm aggregate of fibrofatty tissue. The specimen is difficult to dissect secondary to extremely matted lymph nodes. Dissection reveals 5 candidate lymph nodes ranging from 1.1 x 0.4 x 0.2-5.5 x 2 x 0.8 cm. The specimen is entirely submitted as follows:

- F1-F4: One lymph node candidate
- F5-F6: One lymph node candidate
- F7: Three lymph node candidates
- F8: Remaining fatty tissue

G. The seventh container is additionally identified as, "right pelvic lymph nodes". Received fresh and placed in formalin is a 19.1 g, 7.8 x 3.6 x 1.2 cm aggregate of tan-brown lobulated fatty tissue that is heavily matted and dissected to reveal 8 candidate lymph nodes ranging from 1.4 x 0.6 x 0.2-5 x 2 x 1.1 cm. The specimen is entirely submitted as follows:

- G1: Three lymph node candidates
- G2-G3: One lymph node candidate
- G4-G5: One lymph node candidate bisected, per cassette
- G6-G7: One lymph node candidate
- G8-G14: One lymph node

H. The eighth container is additionally identified as, "left distal ureter". Received fresh and placed in formalin is a 5.8 cm long by 0.4 cm in diameter tan-white tubular structure that is oriented with a stitch on the non-margin side. The non-margin side is inked blue and the margin side is inked black. Representative sections are submitted as follows:

- H1: Margin side en face (black)
- H2: Non-margin side en face (blue)
- H3: Middle sections

I. The ninth container is additionally identified as, "right distal ureter". Received fresh and placed in formalin is a 2.3 cm long by 0.4 cm in diameter tan-brown, tubular structure that is oriented with a stitch on the non-margin side. The mild non-margin side is inked blue. The margin side is inked black. Representative sections are submitted as follows:

- I1: Margin side en face (black)
- I2: Non-margin side en face (blue)
- I3: Middle sections

Intraoperative Consult Diagnosis:

A. Peritoneal nodule: "Ovarian tissue, with benign cyst" by Dr. [] on and reported to Dr. [] at

E. Left pelvic lymph nodes: "Positive for carcinoma" by Dr. [] on and reported to Dr. [] at

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HPA Discrepancy		☒
Prior Malignancy History		☒
Dual/ Synchronous Primary Sites		☒
Case is (circle):	☒ QUALIFIED / ☐ DISQUALIFIED	
Reviewer Initials	Date Reviewed: 7/9/13	